

of this person have I noticed?

How do those parts behave? Make a list.

Intrapsychic Work in Couple Therapy: Going Inside

At various points in IFIO couple therapy, one partner bears witness as the other explores their inner experience. We typically do this individual work within couple therapy for one of these reasons: (1) one partner has a part who is highly charged and won't unblend, (2) patterns of behavior appear entrenched, or (3) negative childhood experiences or shame about having needs begin to surface. While unconventional, individual work in the context of couple therapy can be deeply healing. When a person attends to their wounded, burdened parts with compassion, they re-turn to their partner feeling safer, calmer, braver, and, thus, more available. The inner security that comes from accessing the Self sets the table for acknowledging how negative beliefs (e.g., "I'm unlovable") have impacted their relationship and their partner.

Meanwhile, the witnessing partner sees that vulnerability while also seeing their loved one's strength, resilience, and presence. In many cases, this shared experience challenges the couple's view of each other as just the "wounder" or the "caretaker," and gives them freedom to explore what they actually need and want in their relationship today. When intimate relationships become challenging, implicit memories based on preverbal experiences will begin to surface. We aim to bring implicit memories into awareness so that, over time, the couple can change their perceptions, feelings, and thoughts about themselves and each other (Badenoch, 2008).

In addition to showing partners how childhood experiences influence adult behavior, there are other benefits to individual work in the context of couple therapy. First, the partner who is engaging in self-exploration promotes secure attachments between the Self and their parts, helping parts heal from negative beliefs acquired during childhood. Second, the witnessing partner takes an active role in their partner's healing process by listening with empathy and compassion to painful childhood

dilemmas. When one person is inside witnessing a wounded, vulnerable part with deep love and caring, and their partner holds that vulnerability with love and caring as well, a positive cycle of healing ensues. The inclusive nature of the IFIO model invites relational healing inside and out.

GOING INSIDE

Step One: Contract with the couple and let them know that one person will be going inside while the other serves as a witness.

Step Two: Support the witnessing person to unblend so they can listen and stay present. Remind them that you are there to help if they become afraid or reactive.

Step Three: Gently and respectfully encourage unblending, and help the client establish a compassionate connection with themselves internally. Pay close attention to protectors in both partners to maintain a safe working environment, and continually check for ANS activation.

Step Four: Unburden exiles if appropriate and time allows.

Step Five: Foster and support interpersonal connection and co-regulation.

GOING INSIDE

Mark and Mateo, the couple we introduced earlier who were seeking therapy to improve communication challenges surrounding sex, were doing well with practicing safe, respectful communications, as well as speaking for protective parts and scared young exiles. They had learned to wait before trying to solve problems. They had learned the value of speaking and listening skillfully, which strengthened their understanding of themselves and each other and led to more closeness. However, a small voice inside Mateo continued to ask for help, which warranted additional inner exploration. The following dialogue illustrates how the therapist helped him attend to that part.

Therapist: At the end of the last session, Mateo, we decided to listen to the voice who speaks of being broken and damaged. Do you remember?

Mateo: I do, and I took your advice and didn't minimize it when I noticed it this week. That was challenging, though, because I find it embarrassing.

Say more.

Therapist: Thinking about it and then trying to speak about it seems weak. So vulnerable. I have this strong feeling that I'd rather not.

Therapist: You have parts who judge this voice and push it away?

Mark: [*chiming in*] Yes, he does! Even I notice how much he dislikes this part. Not that I don't get it.

Mateo: Listening to my negative headspace... It's just not something I did before having cancer.

Therapist: How do you feel about listening for a little bit now? Let's ask all parts who judge this voice if they're willing to let you listen.

Mateo: I don't feel much resistance at the moment. Let's try. [*closes his eyes*]

Therapist: I'm just going to check in with Mark. [*to Mark*] Are you available? Heart open? [*he nods*] If for any reason either of you starts to feel uncomfortable, activated, sleepy, or just unable to stay present, please let me know.

Helping Mark's parts unblend

Mark: I will.

Mateo: Okay.

Therapist: Ready, Mateo?

Mateo: I was just remembering how important it felt to be invincible when I was a boy. I got beat up a few times. And coming out in high school was a nightmare. So I made it a mission to be strong, healthy, desirable, and unstoppable.

Therapist: Could we name that as a part of you?

Mateo: Yes! A great part of me.

Therapist: And who does that part protect?

Mateo: I see where you're going. The weak, vulnerable boy who got bullied.

Naming the exile

Therapist: That's right. Can you see him in your mind's eye?

Mateo: [*leaning back, eyes closed*] Yes.

Therapist: How do you feel toward him?

Checking for unblending

Mateo: I feel sorry for him.

Therapist: Is that pity or concern?

Checking again for unblending

Mateo: Not pity. Kind, open. He's small, which makes him a big target.

Therapist: For whom?

Mateo: His brothers and other kids.

Therapist: Is he aware of you?

Fostering the Self-to-part connection

Mateo: Yes.

Therapist: Are you available to listen to him?

Mateo: I am.

Therapist: Let him know.

Developing the relationship

Therapist: [*connecting with Mark*] Are you able to listen? [*Mark nods*]

Mateo: Being bullied sucks.

Therapist: Can he tell you or show you?

Mateo: [*nods*] He's showing me.

Witnessing

Therapist: Is that okay?

Mateo: It's a flood. I feel overwhelmed.

Therapist: What does he need from you so he can dial the intensity back?

Mateo: He's afraid I won't stay with him. He thinks if he slows down, I'll push him away.

Therapist: Other parts would like to push him away? Have pushed him away before?

*Naming protectors who feel threatened by
the exile*

Mateo: It's true.

Therapist: What do you say to him?

Mateo: [voice soft] I'm listening. I'm right here.

Therapist: How does he respond?

Mateo: The images are slowing down.

Therapist: Can you stay with him?

Mateo: Yes. [the therapist looks over to Mark, who gives a thumbs up]

*Checking in with the witnessing partner
periodically to maintain connection*

Therapist: What are you noticing now, Mateo?

Mateo: So much sadness and anger. He's upset. Everyone seemed to need a scapegoat—someone to beat up, put down, hurt. Where was the help? He's asking why he wasn't protected.

Therapist: What do you say?

Mateo: [shaking his head] I'm so sorry.

Therapist: Is there a specific incident he needs to show you?

Mateo: He's showing me how his brothers stuffed him in the dryer at the laundromat. [Mark grimaces] He couldn't stop them. I remember the fear that they would turn it on and the nausea. They didn't, thank God. He threw up in the wastebasket when he got out and they teased him about that.

Therapist: What did he need from an adult then?

Mateo: To get him out of there and set his brothers straight.

Therapist: Can you do that for him?

Mateo: I can.

Therapist: What does he want you to say or do that he needed a grown-up to do then?

Mateo: [*nods*] He's telling me. I'm doing it. [*a few moments of silence*] I'm speaking to his brothers.

Therapist: What did he conclude about himself after that?

Mateo: He felt weak, broken, incapable, worthless.

Therapist: That's a lot for a young boy.

Mateo: This is so sad!

Therapist: How are you feeling toward him now?

Mateo: I see what happened. He didn't deserve any of that. I want to take care of him.

Therapist: Does he know?

Mateo: He does.

Therapist: Where are you both now?

Mateo: He's here with me in the present.

Spontaneous retrieval from the past to the present

Therapist: Would he like to unload those beliefs?

Inviting the exile to unburden, if he's ready

Mateo: I think that's all we can do for today. I think I just need to be with him.

Not ready to unburden yet

Therapist: Sounds good. Will he stay with you?

Honoring the request to stop

Mateo: He's with me. [*looks at Mark*]

Therapist: Can he see Mark?

Mateo: Yes. He sees that Mark's heart is open. That helps.

Mark: Wow, Mateo! I've heard stories about your brothers. I know them now, of course. But you've never mentioned the dryer. Unbelievable! I'm so sorry.

Mateo: No wonder I needed to be strong.

Connecting the past to the present

Therapist: And we'll come back to that.

We only do individual work in a couple session if we expect the witnessing partner is capable of being truly available. The witnessing partner helps the other regulate and stay Self-led. The therapist stays connected with both partners and checks in with the witnessing part periodically, either by eye contact or with words. When the client's Self and the witnessing partner extend compassion to the exile together, the result is a *relational unburdening*. Although relational healing is powerful, the timing must be right. On the other hand, sometimes we want to see the partners individually. We suggest individual work for partners when:

1. Couple therapy isn't safe due to emotional or physical violence.
2. The couple asks that each partner have time with you alone.
3. One or both partners prefer to be seen separately to discuss sex or sexuality.

Some therapists routinely meet with a couple once and then see each partner individually for a session before bringing them back together. Some never do this. Yet others decide on a case-by-case basis. In IFIO, we have no particular recommendation. Our primary focus is on the couple and their relationship. We maintain this focus when we see partners alone, whether once or for several sessions. Every practitioner must assess the situation at hand and make this decision on their own. However, if you do decide to see partners individually, you'll want to avoid these common pitfalls:

1. Forgetting to address how individual sessions fit their contract for the therapy.
2. Neglecting to clarify your policy on secrets.
3. Failing to clarify that individual sessions will focus on supporting the relationship and that you are a therapist for their relationship.

4. Causing an imbalance in the therapeutic alliance by seeing one person and not the other.
5. Causing an imbalance in the therapeutic alliance by making one partner the identified patient and agreeing to see this partner individually.

Talking with the couple ahead of time about the reasons for having an individual session and establishing guidelines regarding secrets creates safety. For example, some therapists keep strict boundaries by telling clients they will not keep secrets. Others invite secrets with the understanding that they will help the client reveal these secrets in the context of the therapy. Others feel comfortable holding a secret. We encourage all IFIO therapists to listen to their parts on this issue and maintain integrity.

Shame as Shaming and Shamefulness

People shame each other both inadvertently and purposefully in relationships. In IFIO, we view external shaming as a well-disguised attempt to distract from inner shaming and feelings of shamefulness. Therapists can orient to the active, viral nature of shame by doing three things:

- First, by talking with clients about parts.
- Second, by speaking and writing about shame as an action (shaming) or a state of being (shameful). Viewing shame this way steers us to attend to the internal relational drama that stems from attachment injuries in childhood. In particular, children who feel unlovable or worthless in response to being shamed develop protective parts who feel compelled to take action. Ironically, proactive protectors tend to do a lot shaming inside while reactive protectors rebel by doing the opposite and behaving shamelessly or bullying others.
- Third, by befriending protectors who shame, inside or out. Our goal is to unburden exiles and free these protectors.

SHAMING AND SHAMEFULNESS

The following dialogue illustrates how these guidelines affected a session between one couple, Michaela and George. This cisgendered, heterosexual pair—one of whom (Michaela) was European American, the other of whom (George) had a Jamaican grandfather—came in to explore the issue of shame. Michaela mentioned feeling a lot of shame when her partner, George, would say things about her to other people, even positive things.

Michaela: I don't like being talked about. I feel exposed.

Therapist: Were you aware of that, George?

George: Maybe a little. I didn't know it was a big deal.

Michaela: Truthfully, I haven't said much. I am always careful of his feelings. And then another part gets mad because I'm tiptoeing around.

George: Who says you have to tiptoe around?

Therapist: Let me jump in here. [*to Michaela*] You have a part who is careful not to shame him?

Michaela: When he feels shamed, he gets mad. Like now. That's why I don't say anything.

George: I'm not mad! You're always telling me what I feel.

Therapist: Can I interrupt? [*they nod*] This dynamic is important. I'm going to sum up what I just heard. You, Michaela, have a part who feels exposed if George talks about you, and a part warns you not to say anything to him because it fears he will feel shamed and get mad, is that right? [*she nods*] And then you have another part who thinks you're tiptoeing around his anger and gets mad at him. Is that right?

Michaela: And mad at me.

Therapist: I see. And then what happens?

Michaela: I criticize him for something.

Therapist: And when she criticizes you, George, what do you do?

George: I get mad.

Therapist: You have a part who gets mad. What happens just before this part gets mad?

George: I feel like a bad little boy.

Therapist: So you have a part who feels he's being told he's a bad little boy and a part who gets mad about that?

George: Yes.

Therapist: And do you have any parts inside who tell you that you're bad?

George: Yes. I have a part who says Michaela is right.

Michaela: You do?

George: Yes. But I don't agree with it.

Therapist: So one part tells you that her critic is right and another part disagrees. If I can just keep going with this for a moment? [*they nod*] I'm hearing that you both have parts who do some shaming inside and some parts who feel ashamed. And you've also identified parts in each other who do some external shaming—of each other. Is that right? [*they nod*] Is this news to you?

Michaela: Well kind of. I never thought of it this way. [*with a chuckle*] That's a whole lot of shaming going on!

Therapist: Indeed.

At this point, the therapist had summed up the shame cycle and pointed out that it was active in both partners. Both Michaela and George seem relieved after the therapist's summation. When Michaela chuckled, George leaned back in his chair.

George: Any hope for us?

Therapist: Yes. All this shaming, shamefulness, and anger may seem complicated. But it's common, and we can help your parts. Would you be willing to do a little inner inquiry? [*they nod*] Go inside and ask your shaming parts this question: "If we could help the parts who have been hurt so their feelings of shamefulness didn't pop up all the time, would you still need to criticize?"

George: No.

Michaela: My part says no too. But it doesn't believe anyone can change the past.

Asking protectors this hypothetical question is very important. First, it asserts that we can resolve the underlying problem that motivates the part's behavior. Second, it invites the part to think about its own future: What would happen if it had no motive to do this job? Once a protector names its problem (e.g., an exile who feels shameful), and the therapist suggests that the exile can be helped to feel better, protective parts often object and insist it's not possible. Since protective parts have never been able to resolve the underlying problem of shamefulness—and never will be able to—we validate their concern. At the same time, we assert that we do actually have a way of helping the exile.

Therapist: Your critic doesn't have to believe in a good outcome. It just has to be willing to try something new. Is it?

Michaela: Tentatively.

Therapist: Fair enough. So let me ask this question of you, Michaela, and then I'll ask George. Which came first, the parts who do all this shaming inside and out, or the feeling of being shameful?

Steering them to consider the origins of the burden that keeps their inner shame cycle going

Michaela: *[after a few moments of thought]* My mother is cold in a way. A lot of the time when I was a kid, she was sweet and fun, and then she would freeze me out with this look of contempt, and I never knew why. I couldn't see it coming. She would get angry and just walk away.

George: Her mother is crazy.

Michaela: I hate to admit it, but the critical voice inside *is* my mother.

Therapist: And the one who feels shameful?

Michaela: Is a little girl. I see her.

Therapist: So after her mother shamed her, another part took on the job of criticizing her inside? *[Michaela nods]* What does the critic want for her?

Michaela: Sounds silly when I hear it, but it says it wants her to be safe.

Therapist: Is this strategy working?

Michaela: No.



As this vignette illustrates, relational injury in childhood sets the stage for the relational fears of adults' protectors. Illuminating this inner shaming cycle helps partners connect the dots backward to the roots of protectors' fears, which include not only the original experience of being shamed, but the internal shaming that followed. When parts unblend and the Self is available, partners notice and can take responsibility for their own fierce internal shaming. If their partner is also shaming them, they become less vulnerable to it and more Self-led in response. If their partner is not shaming them, they can notice and take responsibility for the ways in which their protectors project.

The goal in IFIO is to open the door for couples to be different from each other and to have conversations that might be difficult with clarity, confidence, and courage. When they realize they have sufficient inner resources, they take relational risks more freely. Therefore, IFIO prioritizes assessing and healing the toxic effects of having been shamed in the past. In addition, we maintain that therapists can make no better investment than becoming knowledgeable about—and unafraid of—their own shaming and shamed parts. Therapists must notice any of their own parts who feel overwhelmed or want to avoid the client’s shaming protectors or shameful exiles. In session, therapists can help these parts unblend by promising to return later. When shaming and shamed parts no longer evoke shamefulness in us, we become helpful.

In order to heal shaming and shamefulness in couple therapy, we make a clear contract for taking an active role as the therapist, and we seek permission to be directive (e.g., “I am going to slow you down”). We also reiterate the couple’s shame cycle many times during a single session and over time throughout treatment, making sure to validate each partner’s needs at the same time. Once the couple’s shame cycle has been identified, we help each partner’s shaming, blaming protectors unblend by focusing on the body and asking how the client feels toward that sensation. Shamefulness in the body is visible through a client’s posture, eye contact, and tone of voice. We validate protectors by appreciating their intention to keep the client safe and for their efforts to help—and, at the same time, we validate the needs of wounded exiles. When we reach childhood events that had lasting sequelae in the form of extreme feeling states and burdensome beliefs, we can then release the client’s protectors, as well as their exiles, in the presence of their partner.

HEALING THE WOUNDS OF SHAMING

The following case illustrates how shame wounds can be healed with the support of a partner. Gwen and Wyatt, a cisgendered, European-American, heterosexual couple in their late forties, came to therapy after their twin boys left home for college. They reported having focused on raising their boys and working to sustain a good lifestyle over the last 18 years. In the process, however, they had lost a sense of emotional and physical intimacy, and their relationship often felt unpleasant to both. Gwen had a habit of trying to get Wyatt’s attention by belittling and shaming him. His protectors responded by trying to “get things right,” but when that failed to quell her, they would stonewall and withdraw. At this point, the couple avoided conflict and lapsed into disconnected conversations, which left them feeling dissatisfied and hopeless. In therapy, they wanted to break the impasse and reconnect.

To do so, they learned about their protective impulses (most notably avoidance) and their emotional needs for acceptance and love, including how conflict arose when they were afraid these needs would not be met. Hypothesizing that their reactivity was linked to early experiences of shaming, the therapist began—slowly and respectfully—to support them in deepening their inner work. As the following dialogue with Wyatt illustrates, not all shame cycles involve rage and blame. Of course, Gwen also needed to explore her angry and vulnerable parts, which the therapist did at another time.

Gwen: *[frustrated]* I'm so tired of the same old dynamic day after day. You just don't want to be with me.

Wyatt: What are you talking about? I'm always trying to be with you. I try to figure out what you need. But all I know is that you're angry.

Gwen: That's not the same as being together. That's like... vigilance. You watch and wait.

Therapist: *[to Gwen]* So you have a frustrated part who hopes for more connection with Wyatt. Right?

Naming parts and needs right away

Gwen: That's right.

Therapist: *[to Wyatt]* And you have parts who try to connect with Gwen but don't succeed and experience her as angry. Am I right?

Checking for agreement

Wyatt: Yes! She's always angry. I can't get it right no matter how hard I try.

Therapist: You both recognize this pattern? Gwen has a part with a need for connection and a frustrated part who speaks for that need, while Wyatt has a part who tries and tries to connect but never feels successful. *[to Wyatt]* And then what?

Identifying a recognizable pattern of protection

Wyatt: I go in my cave.

Therapist: [to Wyatt] And as you go in your cave, Gwen gets frustrated. And as she expresses frustration, you back away more. And the cycle continues until what happens?

Wyatt: I give up and disappear entirely.

Therapist: What does Gwen do then?

Checking for awareness of impact

Wyatt: She disappears too, and we both feel awful.

Gwen: [nods in agreement] Like, why bother trying?

Therapist: This is your pattern. Shall we look at what is happening under your frustration and hopelessness to understand why this pattern is hard to break?

Reassuring exiles that we will look beyond protectors

Wyatt: Sure. [Gwen nods]

Therapist: Gwen, it sounds like your frustrated part is here right now. Correct?

Gwen: Actually, I want to start over. Speaking from my frustration doesn't work, right? Could I start over?

At this point, Gwen caught her protector being critical. She paused and requested an opportunity to speak *for* that part instead of *from* it. Recognizing that her protector was hurting Wyatt and choosing to try a different behavior was relationally reparative.

Therapist: [to Wyatt] Are you available for Gwen to try something different?

Seeking permission

Wyatt: [smiling] Sure!

Gwen: Look, it's true, I have needs that aren't being met, and I am the first to admit I'm not very skilled at asking. But ... this part of me wants you to know that your part who tries so hard to figure out what I want and need is not getting it right. I don't need caretaking. And when I feel frustrated about that, you run away and I'm completely alone.

Therapist: [to Wyatt] What's happening as you listen?

Inviting a U-turn. "What's happening?" is a more effective U-turn invitation than asking, "How do you feel?"

Wyatt: I'm honestly confused. I don't know what to do.

Therapist: You have a part who is trying hard to *do* something. Say more.

Naming the part and its activity

Wyatt: Look, my feeling is that I'm always in trouble. There must be something else I'm supposed to do, right?

Therapist: What is the hope of the part who tries to *do* something?

Fleshing out its hopes (or fears)

Wyatt: That I'll get it right and understand what's going on.

Therapist: So that Gwen will be happy? [*he nods*] And then what?

Deepening the inquiry

Wyatt: [*after a long silence*] I don't know... She'll like me. I'll feel successful. I won't be a failure in her eyes.

Therapist: [to Gwen] Were you aware of this?

Gwen: Not exactly. Not what Wyatt is saying right now.

Therapist: [to Gwen] Would you like to know more?

Gwen: Yeah!

Therapist: [to Gwen] If Wyatt is available to do some exploration with me right now, are you available to listen?

*Looking for a genuine yes, where no parts
are objecting*

Gwen: I am.

Therapist: [to Wyatt] Would you be willing to do some exploring with me right now and get to know this part who feels like a failure?

Wyatt: Okay, I guess.

Therapist: [to Wyatt] Sounds like some parts have concerns. Let's check in with that before we go anywhere.

Wyatt: Oh well. I'm not fond of this "exploring parts" stuff.

Therapist: I understand. Let me reassure your wary parts. You are in charge here, and you get to say what you are available for. If you're willing to do this, we will check in regularly with your concerned parts, and they can interrupt at any time.

*Inviting protectors to stick around and do
their job if they feel the need*

Wyatt: That sounds good.

Therapist: [to Gwen] You have agreed to witness Wyatt's inquiry today. I want to check with you. Is that a genuine yes? Are there any parts who feel hesitant?

*Helping activated parts in the listening
partner to unblend*

Gwen: Yeah, I'm sure. I'm truly curious.

*Listening for the "C" words that speak of
the client's Self, like curiosity, caring, and
compassion*

Therapist: [to Gwen] If you need help, I'm here.

Reassuring the client and offering ongoing help

Therapist: [continuing with Wyatt] As I listened, I heard you speaking of many parts. A vigilant part who tries to take care of Gwen, a part who withdraws to avoid conflict, and a part who doesn't want to fail in Gwen's eyes.

Naming parts to differentiate them and support unblending

Wyatt: Sounds right. I'm feeling defeated right now.

Therapist: What if we started right there? The defeated feeling.

Wyatt: It's heavy.

Therapist: Is it okay to focus on that? [he nods] Is the feeling familiar?

Wyatt: All my life. I try and try but never quite get things right. Except maybe parenting. I think I've done a good job with the kids. Oh, and I guess I've done okay in my career. But in relationships, I'm a loser.

Therapist: There's a judging part. It thinks you're a loser in your relationship with Gwen?

Naming inner shaming

Wyatt: [angrily] Obviously Gwen thinks so. She complains enough.

Revealing his external shaming part (the "outer judge")

Therapist: So there's a part who feels like you're a loser and a part who judges Gwen when you feel that way. Would these parts move back slightly so you can focus on the one who feels so bad?

Wyatt: Being pissed off feels better.

Therapist: That makes sense. Can I ask you a question? [*he nods*] Does it make sense that the parts who are judging you and Gwen are trying to protect you from feeling like a loser?

Wyatt: Sure! Who wants this?

Therapist: [*to Gwen*] Are you with us?

Gwen: I am.

Therapist: Okay, Wyatt, let's ask these judging parts if they are willing to soften and let you try something new.

Helping protectors unblend

Wyatt: [*closes his eyes*] I can feel that uncomfortable tension between just wanting to make sure Gwen is okay and wanting to withdraw from her.

Therapist: You spoke of those parts earlier. So here they are.

Wyatt: Yes.

Therapist: I can assure them that Gwen is doing fine. She's here with me feeling curious. Would they give you a little room so we can go on checking in with the judging parts?

Wyatt: Okay.

Therapist: Are they willing to let you try something new?

Wyatt: Reluctantly. Oh, okay. As long as they don't have to go away.

Therapist: What do you notice now?

Wyatt: A lump in my stomach. This is so familiar. My mother. Always needing something that she wasn't able to give herself. I was so overwhelmed. And where was my fucking father!

Therapist: Where was your father?

Wyatt: Just around being a bastard. He loved humiliating me. He was always envious!

Therapist: Here's a part who's angry with your father. Who needs your attention first? The angry one or the one who feels like a loser?

Wyatt: The loser.

Therapist: How are you feeling toward that part now?

Checking for blending

Wyatt: I feel sorry for him.

Therapist: In kindness or pity?

Wyatt: [*putting his face in his hands*] Some of both. [*the therapist turns toward Gwen to make eye contact*]

*Staying connected with the partner
throughout the session*

Gwen: This is so sad!

Therapist: [*to Wyatt*] Did you hear Gwen?

Wyatt: Yes. But I don't want pity.

Gwen: I don't pity you! I'm listening. Remember I knew your father well. He wanted to destroy you.

Gwen's statement at this point in the dialogue signaled that she was unblended and that her heart was open. This kind of active, attuned listening creates safety and deepens the process.

Wyatt: [*weeping*] Yes. Thank you. That helps. But I need to get out of this. [*his eyes are still tightly closed*] It's too much!

Therapist: We can take you out of this any time. But can we first check on who inside is feeling overwhelmed right now?

*Providing reassurance and asking for
permission*

Wyatt: All those parts we asked to wait.

Therapist: What's happening with them?

Wyatt: I'm so ashamed.

Therapist: If we could free that part from feeling shameful, would it help all your other parts?

Wyatt: It would be a huge relief.

Therapist: Okay. Do we have everyone's permission to do that?

Wyatt: Okay.

Therapist: What I heard, Wyatt, is that your mother put a lot of pressure on you to take care of her, while your father humiliated you. Did I get that right?

Wyatt: That's it exactly.

Therapist: Can you see the boy who was pressured and shamed in this way?

Wyatt: Yes.

Therapist: How do you feel toward him?

Checking for unblending

Wyatt: I care about him.

Therapist: Is it okay to stay with him? [*Wyatt takes a big breath and nods*] How old is he?

Wyatt: Many ages.

Therapist: How far back can he take you?

Wyatt: To age three.

Therapist: Can you see the three-year-old? [*Wyatt nods*] Hear him? [*Wyatt nods*] Feel him?

Wyatt: I feel him.

Therapist: How do you feel toward him?

Checking for unblending again

Wyatt: I'm open.

Therapist: What did he need back then from an adult that he didn't get? [*making eye contact with Gwen, who nods*]

Staying connected with the partner

Wyatt: He needed to be a child, not a thing!

Therapist: Of course he did. Can you let him know you understand?

Wyatt: [*opens his eyes and looks at Gwen*] I am afraid of getting it wrong. I guess that makes sense. There was so much at risk. [*Gwen nods*]

Therapist: Stay with him a little longer if you can. [*Wyatt closes his eyes*] How does he respond to you?

Wyatt: He's glad I'm here.

Therapist: What did he come to believe about himself?

Wyatt: I get it. He felt all wrong and worthless.

Therapist: Is he ready to let that go? [*Wyatt nods*] What's happening in your body?

Checking for ANS activation

Wyatt: I feel weirdly relaxed.

Therapist: Where is he now?

Wyatt: Well... he just wanted to be with me, so he got out of there. He's with me.

Therapist: Is he ready to let go of the burden of feeling wrong and worthless? [*Wyatt nods*] How would he like to do that?

Wyatt: [*weeping*] He's so young. I'm holding him just like I hold my boys. It's draining out of him. Is that okay?

Therapist: Is it okay with him?

Wyatt: Yes. It's good. That's good. He's good.

Therapist: What does he need now?

Wyatt: To sleep in my arms where he is safe. He just wants to *be*—you know *be* with me.

Therapist: How are the judgmental parts and that vigilant caretaker doing now?

Wyatt: Relaxed—for now.

Therapist: Would they like to have the opportunity tell their stories sometime?

Wyatt: I think so. Let's see.

Therapist: Yes. Let's see what they need. They can let you know. When it feels right, make your way back. Take your time.



After Wyatt made loving contact with his wounded boy, the couple spent the rest of the session making sense of how chronic shaming in childhood had caused Wyatt to believe he was shameful, which led to his prominent coping strategies and affected their adult life together.

When the Therapist Makes a Relational Mistake*

Most clients are aware that the therapist is in a position of authority, which we define as the ability to influence and act as a resource (Barstow, 2005). In order to deserve authority, we need more than good intentions. We need to embrace our role with clarity, concern, and compassion. We need to understand the power differential within the therapeutic relationship and be sensitive to the ways in which our mistakes impact our clients (Barstow, 2005). All close relationships suffer misconnection, misattunement, wounding, and conflict, including the therapeutic relationship. Mishaps and mistakes are an inevitable part of being in a relationship.

Our job is to take responsibility for our role and our actions and make corrections as needed. We need to model how to restore trust, renew connection, and strengthen relationships. While mistakes are context dependent, some are common. Here are a few examples. The therapist has a part who:

- Engages the client's protectors in a power struggle
- Is misattuned for some reason and gets it wrong—and the client experiences empathic failure
- Commits a microaggression due to implicit bias
- Is not clear about something, like money, scheduling, or what they can and cannot provide
- Gets defensive when a client gives them feedback
- Talks about their own experience without getting permission from the client first
- Doesn't recognize that the therapeutic relationship is not mutual
- Fails to address erotic transference or countertransference appropriately

Even when we have positive intentions, we can have a negative impact on our clients. However, we are not likely to learn the true impact of our therapeutic interventions—and whether we have caused a relational rupture—unless the client's parts feel free to express themselves. We create this level of comfort by listening carefully, not defending ourselves, and staying curious about our impact. In addition, we need to be mindful of waiting to say what we really mean until after the client feels heard (“Is there more you want to speak for?”) (Barstow, 2005) and the client's protectors have been willing to unblend.

Good interpersonal boundaries reduce the likelihood of mistakes and mitigate their impact when they do occur. Good boundaries create an atmosphere of safety and are essential for the therapeutic alliance. As the person with the most resources in the relationship, we are in charge of setting, communicating, and maintaining boundaries. Although our clients may help to create these boundaries and will have their own feelings and issues regarding boundaries, boundary maintenance is our job. For example:

Iris: My *no* has never meant *no* to anyone. I've been violated many times.

Therapist: Your *no* will mean *no* with me. We will explore any part who says *no* until we both understand its concerns and it feels safe here. I am very sincere about this. Deal?

Iris: Deal!

Even with positive intentions and good boundaries, though, relational mistakes can still impact the therapeutic relationship. Making repairs signals to the client that their safety is paramount, their reality is valid, and the power hierarchy is out in the open. Rather than making a quick apology—which can be used to avoid conflict and side-step difficult conversations—we have to own the mistake and make the repair sincere. By taking full responsibility for our actions without being defensive, self-deprecating, or placating the client, we model how to be congruent, present, and unblended. If the client has a part who tries to rescue you from a relational mistake, ask them to help fearful parts relax back so they can speak for any parts who feel angry, hurt, or confused. Mistakes can also, of course, activate our own critics to go on the attack internally. We may have a protective part who:

- Shames us for not being perfect, which interferes with our ability to stay interpersonally connected
- Activates in response to an exile's shamefulness internally and responds by getting angry with the client
- Believes admitting mistakes means we are a bad therapist
- Feels powerless when we are in a one-down position
- Fears that admitting to a mistake will lead to losing the client's respect and losing influence with the client

To overcome these challenges, we suggest making a commitment to exploring your parts going forward and forgiving any part who makes a mistake. In addition, making repairs is an ongoing process that promotes safety, so we ask clients to give us feedback on a regular basis. Finally, get consultation when you need it.

PHASE 3 OF TREATMENT: ENDING

In the last phase of IFIO, we help couples heal wounds and forgive betrayal, which all couples experience at some point. Some betrayals are small and mundane, like poor attunement. Others, like having an affair, can be very consequential. Repeated, unresolved injuries, great and small, cause protective parts to commit even more resources to guarding the heart. As the relationship erodes, the couple comes to view each other as adversaries rather than supports and resources. To recapture trust, they need a safe way to take responsibility for injurious behaviors. In this last phase of therapy, we unpack the experiences, feelings, and beliefs that couples often have regarding betrayal, apology, and forgiveness.

Repair

Oftentimes, individuals struggle to make amends because their protectors are holding onto a variety of distortions that block repair. For example, they may have protectors who associate the experience of apologizing with:

- Accepting undeserved or unbearable blame
 - o “It takes two to tango. If I take full responsibility, I’ll be letting her off the hook.”
- Encouraging a variety of future dangers, especially being shamed
 - o “When I say that I’m sorry, he uses it against me later.”
- Being chronically deprived of validation in the past, so they insist on being correct now
 - o “I’m right, so why should I apologize?”

When protectors promote these distortions, people tend to move on without hearing each other or without ever